PEDIATRIC SURGERY · LECTURE 3 · QUESTION BANK

CHPS & Tracheo-Oesophageal Fistula

ضيق البواب الخلقي والناسور المريئي الرغامي — بنك الأسئلة

20	6	12
Past exam questions	NCLEX-style	Flashcards

المصادر اللي اتجمعت منها أسئلة المحاضرة دي:

أسئلة سابقة · فاينال (مراجعة أسئلة) · ميدترم 2025 · ميدترم السنة اللي فاتت (Microsoft Forms)

جنب كل سؤال مصدره. لو السؤال اتكرر في كذا امتحان مكتوب كل مرة بمصدرها، وعليه علامة "تكرر". الإجابات كلها في آخر الملف.

اللَّهُمَّ لا سَهْلَ إِلَّا ما جَعَلْتَهُ سَهْلًا، وأنتَ تَجْعَلُ الحَزْنَ إذا شِئْتَ سَهْلًا — صلِّ على النبي ﷺ

Part A · Past Exam Questions

أسئلة الدفعات السابقة

1 In cases of TOF/OA (tracheo-oesophageal fistula / oesophageal atresia): — ميدترم السنة اللي فاتت (Microsoft Forms)

a. It's an emergency surgery to ligate the fistula in case of associated imperforate anus
b. It can be done by thoracoscopy
c. We may do feeding gastrostomy if anastomosis failed
d. All of the above

2 In case of IHPS (infantile hypertrophic pyloric stenosis): — ميدترم السنة اللي فاتت (Microsoft Forms)

a. It needs pyloromyotomy
b. Can be done by thoracoscopy
c. Give the baby blood always
d. Vomiting is always bilious

3 The patient with esophageal atresia with or without fistula may present with all EXCEPT: — ميدترم السنة اللي فاتت (Microsoft Forms)

a. Excessive salivation
b. Gasless abdomen in X-ray in type A esophageal atresia
c. Bilious vomiting
d. Chest complications

4 You receive a baby after birth; how do you suspect esophageal atresia? — ميدترم السنة اللي فاتت (Microsoft Forms)

a. Visible abdominal pain
b. Failure to pass meconium
c. Failure to pass Ryle (NGT)
d. Vomiting

5 Classic symptom of pyloric stenosis in the infant is: — ميدترم 2025

a. Projectile bilious vomiting
b. Projectile non-bilious vomiting
c. Bloody vomiting
d. Vomiting with fever

6 Which electrolyte abnormality is commonly associated with pyloric stenosis? — ميدترم 2025

a. Hyperchloremic metabolic acidosis
b. Hypochloremic metabolic alkalosis
c. Hyperkalemia
d. Hyponatremia

7 Significant treatment of pyloric stenosis: — ميدترم 2025

a. Antibiotic therapy
b. Pyloromyotomy
c. Nasogastric decompression
d. Fluid restriction

8 Which clinical finding is most suggestive of esophageal atresia in a newborn? — ميدترم 2025

a. Excessive oral saliva secretion
b. Diarrhea
c. Hypotonia
d. Seizures

9 Esophageal atresia is most associated with which condition? — ميدترم 2025

a. Down syndrome
b. VACTERL association
c. Turner syndrome
d. Marfan syndrome

10 Which diagnostic test confirms esophageal atresia? — ميدترم 2025

a. Blood test
b. Passage of a nasogastric tube and X-ray
c. Urinalysis
d. Echocardiography

11 In all cases of tracheoesophageal fistula we perform tracheostomy and gastrostomy. (True / False) — فاينال (مراجعة أسئلة)

a. True
b. False

12 CHPS commonly causes bilious vomiting. (True / False) — فاينال (مراجعة أسئلة)

a. True
b. False

13 A child with esophageal atresia is given oral antibiotics. (True / False) — فاينال (مراجعة أسئلة)

a. True
b. False

14 Treatment of esophageal atresia is surgery after preparation. (True / False) — فاينال (مراجعة أسئلة)

a. True
b. False

15 Complete: Tracheoesophageal fistula with imperforate anus is managed as ..... — أسئلة سابقة

.....

16 Complete: The type of surgery in tracheoesophageal fistula can be ..... — أسئلة سابقة

.....

17 Complete: If anastomosis fails in esophageal atresia we do ..... — أسئلة سابقة

.....

18 Complete: The surgery of hypertrophic pyloric stenosis is .....

أسئلة سابقة

.....

19 In tracheoesophageal fistula, vomiting may be bilious. (True / False)

أسئلة سابقة

a. True **b.** False

20 Tracheoesophageal fistula may cause intestinal obstruction. (True / False)

أسئلة سابقة

a. True **b.** False

Part B · NCLEX-Style Practice

أسئلة على طريقة NCLEX (إضافة)

N1 A 5-week-old boy has projectile non-bilious vomiting after every feed and is hungry after vomiting. The nurse suspects: NCLEX

a. Gastroesophageal reflux **b.** Hypertrophic pyloric stenosis
c. Duodenal atresia **d.** Intussusception

N2 Which laboratory pattern is expected in pyloric stenosis? NCLEX

a. Hyperchloremic acidosis
b. Hypochloremic, hypokalemic metabolic alkalosis
c. Hyperkalemia
d. Metabolic acidosis with high anion gap

N3 Select ALL pre-operative priorities for an infant with pyloric stenosis. SELECT ALL

a. Correct dehydration and electrolytes with IV fluids
b. Keep NPO with NG decompression as ordered
c. Monitor intake, output and weight
d. Start large oral feeds immediately
e. Schedule pyloromyotomy after correction

N4 The first sign that should make the nurse suspect esophageal atresia in a newborn is: NCLEX

a. Bilious vomiting
b. Excessive frothy salivation and choking with the first feed
c. Abdominal distension only
d. Failure to pass meconium

N5 Esophageal atresia is commonly associated with which group of anomalies? NCLEX

a. VACTERL **b.** TORCH
c. CHARGE only **d.** Pierre Robin

N6 After TEF repair, the nurse notes the chest tube. Which action is correct? NCLEX

a. Clamp it routinely
b. Keep the drainage system below chest level and monitor drainage
c. Milk it vigorously every hour
d. Remove it if bubbling stops

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

CHPS	Hypertrophy of pyloric muscle → gastric outlet obstruction; 3–6 weeks; boys, first-born.	**CHPS signs**	Projectile non-bilious vomiting, hungry baby, olive mass, visible peristalsis.
CHPS labs	Hypochloremic hypokalemic metabolic alkalosis.	**CHPS surgery**	Ramstedt pyloromyotomy after fluid/electrolyte correction (not emergency).
Post-pyloromyotomy feeding	Clear fluids 4–6 h, then gradual feeds; burp; right side/Fowler's.	**OA/TOF**	Esophageal atresia ± tracheoesophageal fistula; commonest type C (distal TEF).
OA signs	Excess salivation, choking, cyanosis, NG tube can't pass.	**Type A OA**	Pure atresia — gasless abdomen on X-ray.
OA pre-op	NPO, suction of upper pouch, head up, IV fluids, warm.	**OA surgery**	Thoracotomy/thoracoscopy repair; gastrostomy if anastomosis fails.
TOF + imperforate anus	Emergency ligation of fistula.	**VACTERL**	Vertebral, anorectal, cardiac, TE, renal, limb.

Answer Key

الإجابات

1. d — All of the above
سؤال 7 من الامتحان ده مش موجود في الصور. أسئلة diaphragmatic hernia وlymphangioma وNEC اتشالت لأنها مش في قائمة المحاضرات.

2. a — It needs pyloromyotomy

3. c — Bilious vomiting

4. c — Failure to pass Ryle (NGT)

5. b — Projectile non-bilious vomiting
ملف ميدترم 2025 من غير إجابات (الطالب كاتب «حله لوحدك» — كل إجابات الامتحان ده مقترحة ومراجَعة على الملخص.

6. b — Hypochloremic metabolic alkalosis

7. b — Pyloromyotomy

8. a — Excessive oral saliva secretion

9. b — VACTERL association

10. b — Passage of a nasogastric tube and X-ray

11. b — False

12. b — False

13. b — False

14. a — True

15. ✎ — An emergency

16. ✎ — Thoracoscopy

17. ✎ — Gastrostomy

18. ✎ — Pyloromyotomy

19. b — False

20. b — False

N1. b — Hypertrophic pyloric stenosis
CHPS classically appears at 3–6 weeks with projectile non-bilious vomiting and a hungry infant.

N2. b — Hypochloremic, hypokalemic metabolic alkalosis
Loss of gastric HCl through vomiting produces hypochloremic hypokalemic metabolic alkalosis.

N3. a, b, c, e — Correct dehydration and electrolytes with IV fluids; Keep NPO with NG decompression as ordered; Monitor intake, output and weight; Schedule pyloromyotomy after correction
Pyloromyotomy is never an emergency; fluids and electrolytes are corrected first.

N4. b — Excessive frothy salivation and choking with the first feed
Excessive saliva, coughing, choking and cyanosis with feeding; the NG tube cannot be passed.

N5. a — VACTERL
VACTERL: Vertebral, Anorectal, Cardiac, Tracheo-Esophageal, Renal, Limb anomalies.

N6. b — Keep the drainage system below chest level and monitor drainage
Keep the system closed and below the chest; monitor amount and character of drainage.